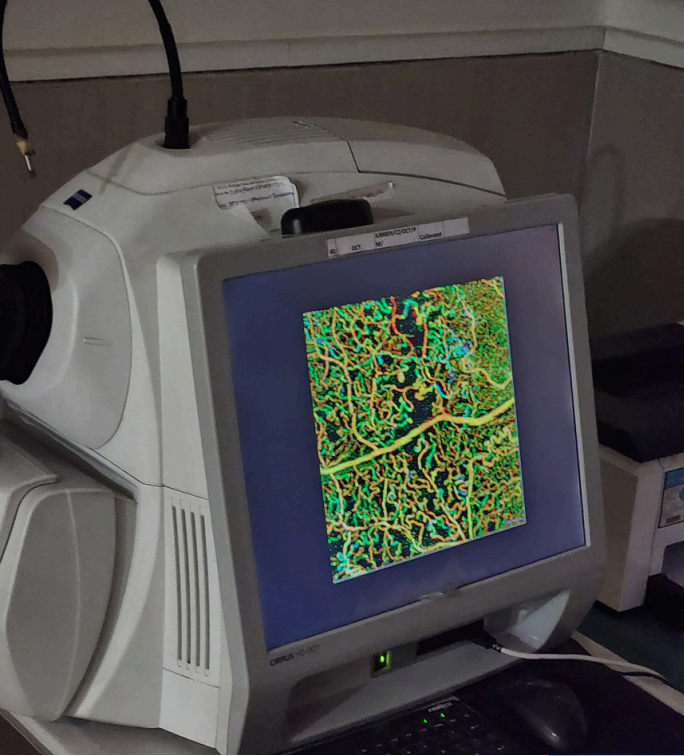


SCOPE AND SCALE OF CLINICAL IMMERSION

- get to know various stakeholders in the eye hospitals //came to know about various schemes // add the various devices that we have seen
- found various needs of huge economic value //
- live validation // met people like surgeons(retina damage they dont know before cataract) //
- the process of the patient entry to end to end data //we get to learn the various workflow of a patient //
- it gave us the users and support system generally not available for the general public who want to solve problems //
- we also learnt about the anatomy and physiology of the ophthalmology then on top various treatments related to them in a short span which is unimaginable without the immersion //

ANSWER THE QUESTIONS THAT ARE THERE IN THE CHAT E, ONE MORE NEED, MAKE THE 2 SLIDES IN 1-3 WORDS ,E THERE IS SOME SCRAPPY TEXT WITH NO REFERENCES OF THE DATA
PREPARE THEM M,
LEARN THE TEXT IN THE BOOK FOR LOW VISION, PREPARE M,
THINK OF DISRUPTIVE IDEATION E,





INSIGHTS FROM IMMERSION



▶ for indian context ,high volume of patients **but** less number of high skilled optimetrists and doctors,need a device that provides high quality information of retina from even screenings by primary healthcare providers. //// 1st point

▶ the devices are very heavy, non ergonomic,imported and expensive mention companies of the devices zeiss ,olympus /// 2 point

low vision is the leading cause of blindness(show data and citations) and pain the patient goes is truly heart touching, hospitals similar to jl raothagi eye hospital

▶ conduct camps every month where they screen around 100 patients per camp for diabetic retinopathy screening remove the numbers (3rd point too heavy and non portable) the devices available (3rd point too heavy and non portable)

